

Sample Patient History and Physical Examination

MedLingua Demo

Patient Information

Patient name Pamela Rogers
Date June 2, 2004
Referral source Emergency Department
Data source Patient

Chief Complaint

Chest pain for one week, associated with shortness of breath.

History of Present Illness

This is the first admission for this 56-year-old woman, who states that she was in her usual state of good health until one week before admission. She noticed the abrupt onset of dull, aching chest pain in the left parasternal area, radiating to her neck. The first episode occurred while working in her garden. She had been working for approximately 45 minutes and felt tired before the pain began.

The discomfort was accompanied by shortness of breath, but no sweating, nausea, or vomiting. The pain lasted approximately 5–10 minutes and resolved when she went inside and rested in a cool area.

Since that initial episode, she has had two additional similar episodes. Three days ago, she had 15 minutes of pain while walking her dog that resolved with rest. On the evening before admission, pain awakened her from sleep and lasted 30 minutes, prompting her visit to the Emergency Department. She has not used medication or other specific measures for relief.

Review of Systems

She reports shortness of breath during the episodes but denies dizziness, palpitations, orthopnea, paroxysmal nocturnal dyspnea, fever, cough, or claudication. She has had no previous chest pain and has never been told that she has heart disease.

Past Medical and Surgical History

- Hypertension diagnosed three years ago.
- Total abdominal hysterectomy with bilateral salpingo-oophorectomy for uterine fibroids in 1998.
- Bunionectomy in 1998.

Medications and Allergies

She is not taking hormone replacement therapy. Penicillin allergy is reported.

Family and Social History

There is a family history of premature coronary artery disease. She does not smoke and does not have diabetes. Her cholesterol level is unknown.

Assessment

1. New-onset exertional and rest chest pain concerning for ischemic heart disease or unstable angina.
2. Dyspnea associated with chest pain; evaluate for ischemia and left ventricular dysfunction.
3. Hypertension with possible vascular contribution.
4. Systolic murmur requiring characterization.

Initial Plan

1. Admit to telemetry and monitor for recurrent or worsening chest pain.
2. Obtain serial electrocardiograms, cardiac biomarkers, complete blood count, metabolic panel, and renal function tests.
3. Consider aspirin and nitrates according to the treating clinician's assessment and contraindications.
4. Obtain echocardiography and cardiology consultation. Consider cardiac catheterization based on the clinical evaluation and non-invasive test results.